TM-002 Clinical Journey Review — Multi-Perspective Assessment

Client: T.M., 22, App State NC | Case: TRAUMA-001 | CSP Pro Bono | Sessions: 3 completed, S4 scripted | Therapist: Jithendran Sellamuthu, C.MH., AHA #007913 | Review date: Nov 6, 2026

1. PRESENTING ISSUES vs. CURRENT STATE

PRESENTING ISSUE (INTAKE, OCT 8)	WHERE WE ARE NOW (POST-S3, OCT 24)	MOVEMENT
1. Childhood trauma "Past traumatic experiences from when I was a young kid"	Fully mapped: absent parents (incarcerated), foster neglect, 3 abusive uncles, internalized blame. Root wound identified as abandonment + alienation. Inner child work scripted for S4.	**Significant** -- from vague presenting to precise clinical target
2. Self-esteem "Feel comfortable in my own skin"	Core belief identified: "I'm not a good person" (internalized uncle blame). Esteemable acts framework delivered. Self-labeling loop mapped (judgment->guilt->label->shame). Toolkit provided.	**Moderate** -- intellectually understood, subconsciously unchanged (per journal)
3. Confidence / assertiveness Voice suppressed in childhood	Protector energy reframe deployed successfully at work. Listener/helper role adopted in MKP group. Not yet tested in high-stakes personal assertion.	**Significant** -- real-world evidence of behavioral change
4. Trust issues "Trusting people that are close to me"	Core belief identified: "People I care about don't actually care about me." Feels alone even in company. Behavioral strategy (incremental trust via service) deployed but subconscious belief persists.	**Moderate** -- correctly diagnosed, not yet treated at subconscious level
5. Anxiety / hypervigilance "Anxiety still runs the show"	Tribe circle + anchor installed (S2). Observer stance sustained ("easier to watch thoughts arise"). Protector energy held under real-world pressure. Circle Therapy scripted but not yet delivered.	**Significant** -- from "runs the show" to functional regulation at work
6. Letting go Holding on = protection strategy	Reframed as learned emotional guarding (known vs unknown principle). Client understands the mechanism. Guard still active in group settings. Needs subconscious work to release.	**Moderate** -- insight achieved, behavioral shift incomplete

2. CLIENT PERSPECTIVE

VOICE: T.M. (CLIENT)

What's working for me:

- The anchor really works. When those angry visitors came at me at the park, I pressed my thumb and finger together and something shifted. I didn't take it personally. That's new for me.
- The observer thing from Session 1 -- watching my thoughts without reacting -- that stuck. I still do it.
- Offering support to others in the group felt natural. I can do that part.
- The "they're struggling" reframe -- I used it at work and it changed how I handled the whole situation.

What's still hard:

- I still feel alone even when I'm surrounded by people who seem to care. It's like there's a glass wall.
- I still catch myself thinking I'm not a good person. Even after doing good things. The feeling doesn't match the facts.
- The group is great but I can't fully open up. I'm verbally there but internally guarded. I don't know how to change that with practice alone.
- The self-labeling -- calling myself an "asshole" when I have a judgmental thought -- I catch it faster now but it still fires.

What I need:

- Something that reaches the part of me that knows the facts but can't feel them. The behavioral stuff helps at the surface. But underneath, the old programming is still running.

3. HMI INSTRUCTOR PANEL REVIEW

VOICE: HMI INSTRUCTOR PANEL

Lisa Machenberg (Wound Identification + Desensitization)

Assessment: Wound identification is excellent. The therapist correctly refined from the broad betrayal/shame formulation to abandonment + alienation as the primary wound. The journal confirmed it: "people I care about don't actually care about me" is textbook abandonment encoding. The phased approach (behavioral first, then inner child) is sound -- Lisa's model supports this: you don't desensitize until you know the exact target.

Strength: The therapist followed the client's material rather than forcing the prepared script in all three sessions. This is correct Emotional-lane practice. Imagery and experience over analysis.

Concern: SMART baselines have been deferred three times. You need numbers before S4 to establish a measurable starting point. Non-negotiable.

S4 script review: The two reparenting targets are precise. "Nobody truly cares" maps directly to the abandonment hypnotic intention: "You will never abandon yourself." "I'm not a good person" maps to shame: "Watch how you talk to yourself." The script correctly uses the client's own words and evidence (river rescue, work incident, group support) rather than generic affirmations. **Approved.**

John Melton (Inner Child + Expectations)

Assessment: The Room Technique preparation is correct. Room is NEW, not from childhood. Adult observes child from current perspective. Agency language throughout. Integration is optional. The script does NOT script the reparenting words -- it lets the client discover them. This is critical.

Strength: The gating on Phase 10 (uncle blame) is good practice. One deep reparenting is a strong session. Don't push for two if the first one was intense. The therapist built this in.

Concern: Watch for intellectualization. This client has done prior psychotherapy. He may try to analyze the inner child experience rather than feel it. If you see him narrating instead of experiencing, redirect: "Stop describing it. Feel it. Where in your body?"

Expectations check: The pre-talk (Phase 3) correctly frames what inner child work IS and IS NOT. Not regression. Not reliving. Observing from adult safety. This manages expectations before trance.

George Kappas (Reprogramming vs. Story)

Assessment: The therapist is walking the line correctly. Sessions 1 and 3 gathered the story. Session 2 delivered experiential programming (tribe circle, anchor). Session 4 targets the PROGRAMMING (two specific beliefs) not the story (childhood narrative). This is the right balance.

Concern: Don't let the client loop in the childhood narrative during S4. You have the story. You identified the beliefs. Now you reprogram them. The script should move from "meet the child" to "correct the belief" without extended story-dwelling. The current script handles this -- maintain discipline in delivery.

Key principle applied: "The issue is not the childhood event. It's the message units that got hardwired." The S4 script targets exactly two message units: "nobody cares" and "I'm not good." Correct.

Saman Nasir (Neuroscience-Informed)

Assessment: The trajectory is neuroscience-sound. Sessions 1-2 activated vagal toning and amygdala calming (progressive relaxation, anchor, tribe circle). Session 3 gathered hippocampal context (the childhood memories that need re-filing). Session 4 performs memory reconsolidation: revisit the emotional memory in trance, add adult perspective, re-file with updated meaning.

Strength: The compound anchor strategy is excellent. Each session layers new neural associations onto the same physical gesture (thumb + forefinger). By S4, one press activates: tribe protection + circle calm + workshop builder + child integration. This is kinesthetic stacking at its best.

Neural pathway note: The post-hypnotic in Phase 12 ("before the belief completes, the adult's voice will be there") is designing a neural interrupt. The old pathway fires, hits the new pathway, and the new one wins because it's more recent and trance-encoded. This is how reconsolidation works in practice.

4. CSP COORDINATING ASSESSOR REVIEW

VOICE: CSP COORDINATING ASSESSOR

Compliance Assessment

REQUIREMENT	STATUS	NOTES
SB 577 Disclosure signed	**COMPLIANT**	Signed Oct 7 via eSignature
Recording Consent signed	**COMPLIANT**	Signed Oct 7 via eSignature
Treatment Plan delivered	**COMPLIANT**	Sent Oct 7 by email
CCH Logs filed	**PARTIAL**	S1 and S2a logs created. S3 log created. Verify all submitted to HMI portal.
Session documentation	**COMPLIANT**	AVS for S1, S2a, S3. CCH logs for S1, S2a, S3. Scripts for S1, S2, S3, S4.

Supervisor consultation	**COMPLIANT**	Case conference guidance from Machenberg, Melton, Kappas, Nasir documented in CCH logs
Client initials in shared docs	**COMPLIANT**	T.M. used in all CCH logs, AVS, portal
Crisis protocol in place	**COMPLIANT**	988 + anchor + email in every AVS and session close
Pro bono fee compliance	**COMPLIANT**	$0 per CSP agreement
SMART baselines collected	**NOT YET**	Deferred from S1, S2, S3. MUST collect in S4.

Clinical Quality Assessment

Strength -- Session adaptation: The therapist adapted from the prepared script in all three sessions to follow the client's presenting material. In each case, the adaptation was clinically justified and documented. This demonstrates clinical judgment, not poor preparation. The scripts were prepared; the therapist chose to follow the client. This is appropriate for Emotional-lane clients and trauma-informed care.

Strength -- Documentation quality: Comprehensive documentation for a CSP case. Scripts, runbooks, AVS, CCH logs, feedback forms, practice cards, email templates, self-esteem toolkit. Well above minimum CSP requirements.

Strength -- Phased approach: The decision to defer inner child work until behavioral coping was demonstrated is clinically sound. The therapist explicitly documented the rationale: "premature deep regression without sufficient external coping capacity risks destabilization." This shows risk awareness.

Concern -- Session count: 3 sessions completed with 1 trance session (S2). The case presents with complex developmental trauma. 6-8 sessions may be insufficient. The therapist should plan for contingency sessions and document the decision framework for discharge vs. extension.

Concern -- Trance-to-talk ratio: 1 trance session out of 3 is low for a hypnotherapy case. S1 adapted to EFT, S3 adapted to psychoeducation. Both adaptations were justified, but the assessor would want to see hypnotic technique delivery in S4 and S5 to validate that hypnotherapy (not just counseling) is being practiced.

Concern -- SMART baselines: Three deferrals is a documentation gap. Without pre-treatment baselines, post-treatment efficacy cannot be objectively demonstrated. Collect in S4 opening. Non-negotiable.

CCH Hour Tracking

SESSION	DATE	DURATION	HOURS	TYPE
S1	Oct 8	90 min	1.5	Assessment + EFT
S2	Oct 16	90 min	1.5	Hypnotherapy (trance)
S3	Oct 24	90 min	1.5	Psychoeducation + planning
Total TM-002			**4.5 hrs**	of 72 needed

Combined with DS-001 (3 sessions = 4.5 hrs): **9.0 CCH hours logged** toward 72 needed. 63 remaining. Hard stop: Dec 10, 2026 (45 days). At 1.5 hrs/session, need ~42 more sessions. **This pace is unsustainable for the hard stop.** Therapist needs additional clients or accelerated scheduling.

5. THERAPIST SELF-ASSESSMENT

VOICE: THERAPIST (JITHENDRAN)

What's Working

- **Emotional-lane delivery is landing:** Permissive, imagery-rich, client-discovers approach is producing real behavioral change. The protector energy reframe transferred to real life without explicit practice instruction -- it integrated subconsciously from the S2 trance experience.
- **Compound anchor strategy:** Each session layers new associations. The anchor is becoming a multi-dimensional tool. By S4, one gesture will contain tribe + circle + workshop + child integration.
- **Session adaptation discipline:** Following the client's material has been correct every time. The prepared scripts serve as safety nets and demonstrate preparation, but the real work happens in the adaptive space.
- **Documentation-as-thinking:** The preparation process (scripts, runbooks, technique docs) clarifies clinical thinking before the session. Even when the script isn't delivered verbatim, the preparation makes the adaptation more precise.

What Needs Improvement

- **SMART baselines:** Three deferrals is unacceptable. Collect in S4 opening. No exceptions.

- **Trance-to-talk ratio:** S4 MUST deliver trance work. Two consecutive non-trance sessions risks the client (and the assessor) questioning whether this is hypnotherapy or counseling.
- **CCH pace:** 9 hours toward 72 in 45 days is behind. Need to accelerate scheduling and/or add clients.
- **Post-session checklists:** Several items from S2 checklist still unchecked (recording sent? tracker exported?). Need to close the loop within 24 hours of each session.

S4 Risk Assessment

Abreaction risk: MODERATE. Inner child work with a client who has developmental trauma (not single-event) carries higher abreaction risk. The client has never been in trance for reparenting work. Mitigation: safe room established first, agency language, gated Phase 10, deepen-don't-count-out protocol, exit signal reviewed.

Intellectualization risk: MODERATE. Prior psychotherapy client. May analyze rather than experience. Mitigation: if narrating, redirect to body sensation. "Where do you feel that?" not "What do you think about that?"

Incomplete processing risk: LOW-MODERATE. If only one reparenting belief is addressed (Phase 9 but not Phase 10), the session is still therapeutically valuable. One deep correction is better than two shallow ones. Phase 10 is explicitly gated.

6. SUCCESS METRICS — EXPECTED OUTCOMES

SMART Baselines (to be collected S4 opening)

MEASURE	EXPECTED BASELINE (S4)	TARGET POST-S6	GRADUATION (S7-8)
Self-Esteem (SE)	3-5/10	5-6/10	6-7/10 sustained
Assertiveness (AS)	4-5/10	5-6/10	6-7/10 sustained
Anxiety/Calm (AX)	4-5/10	6-7/10	7+/10 sustained
Trust (TR)	3-4/10	4-5/10	5-6/10 sustained
Comfort in Own Skin (CO)	3-4/10	5-6/10	6-7/10 sustained
Influence of Past (PI)	3-5/10	5-6/10	7+/10 sustained

Note: "Sustained" = client self-reports the score for 2+ consecutive sessions. These are SUBJECTIVE measures (1-10 self-report), not biometric targets, per HMI instructor rules.

Behavioral Markers (Observable)

MARKER	CURRENT (POST-S3)	TARGET POST-S6
Protector energy in real situations	Deployed once (work)	Consistent deployment across contexts
Self-labeling frequency	Still fires, caught faster	Catches within seconds, replaces with balanced truth
Group vulnerability	Verbally present, internally guarded	Shares something real in group at least once per session
Anchor usage	Uses when reminded	Automatic in triggering situations
Loneliness in company	"Very alone even in the group"	Moments of genuine connection reported
Self-talk quality	Critic dominant	Coach voice present, critic weakened

Graduation Criteria

1. SMART baselines show sustained improvement (2+ consecutive sessions at or above target)
2. Client reports feeling "less alone" in group settings (not zero loneliness, but reduced)
3. Core beliefs ("nobody cares" / "not a good person") no longer fire automatically OR are immediately countered by evidence
4. Anchor is used independently without therapist prompting

5. Client can articulate the difference between the old story and the corrected story
6. Client demonstrates self-regulation in at least 2 real-world triggering situations
7. Client expresses readiness for independence

Earliest graduation: S6 (if inner child work lands deeply in S4 + Circle Therapy in S5 + integration in S6).
Expected graduation: S7-8 (contingency sessions likely needed for complex developmental trauma).

7. EFFICACY OF METHODS AND SCRIPTS

METHOD / SCRIPT	DELIVERED	EFFICACY	EVIDENCE
EFT Tapping (S1)	S1	**Effective**	Client well-received, reported immediate calm. Integrated as grounding tool.
Emotional-lane Induction (affirmations + visual anchor)	S2	**Effective**	Smooth entry, client reported "more steady" post-trance. Consistent protocol established.
Tribe Circle Hypnodrama	S2	**Highly effective**	Client reported "armor or shield." 11 members integrated. Golden light seal produced tangible protective experience.
Thumb-Forefinger Anchor	S2	**Highly effective**	Used unprompted at work (angry visitors). Real-world transfer confirmed.
Rescuer-Helper Reframe	S2	**Highly effective**	Deployed at work: "they're struggling" replaced defensive anger. Therapist noted this typically takes years.
Observer Stance (S1 seed)	S1	**Effective**	"Easier to watch thoughts arise and let them be." Sustained across S2 and S3.
Identity vs. Behavior Separation	S2	**Partially effective**	Intellectually understood. Self-labeling still fires but caught faster. Needs subconscious reinforcement.
Known vs. Unknown Psychoeducation	S3	**Insight delivered**	Client understands the mechanism. Behavioral change not yet demonstrated.
Listener/Helper Strategy	S3	**Effective**	"Easy to offer support." Reduced closed-off feeling. Incremental trust building.
Self-Esteem Flip Theory	S3 (toolkit)	**Not yet tested**	Toolkit delivered, not yet deployed in trance. S4+ will test.
Circle Therapy (fear extinction)	Not delivered	**Pending**	Scripted, merged, ready. Planned for S5.
Inner Child Reparenting	Not delivered	**Pending S4**	Scripted based on journal. Two precise belief targets identified.

VOICE: HMI INSTRUCTOR PANEL

Overall efficacy assessment (instructor panel): The methods that have been delivered are performing well. The tribe circle and anchor compound are the standout successes -- producing real-world behavioral transfer in under 3 sessions. The concern is that the heavier therapeutic tools (Circle Therapy, inner child reparenting) have not yet been delivered. S4 is the inflection point. If the inner child work lands, the trajectory is strong. If it doesn't, the case may need extension beyond 8 sessions.

VOICE: CSP COORDINATING ASSESSOR

CSP assessor summary: Documentation is thorough. Clinical reasoning is sound. Adaptation pattern is justified. The primary gap is SMART baselines and trance delivery frequency. S4 must deliver both. The CCH pace concern is separate from clinical quality -- the therapist needs more clients, not different methods.